



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رباعياً : ربهام سميره صالح حسن
المحافظة : القليوبية
الحى / المدينة : القناطر الخيرية
العنوان : باسوس
رقم البطاقة : 29705281200961 رقم التليفون : 01120871695
تاريخ تسجيل الإستمارة : 04-10-2026
العمر (تاريخ الميلاد) : 28-05-1997 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور
قسم العلاج :
الاورام

[14163]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ريهام سميره صالح حسن National ID: 29705281200961	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 بطاقة تحقيق الشخصية

ربهام
 سمير صالح حسن محمد
 عربية رستهيم باسوس
 مركز القاهرة الخديوية - القنوية

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ر.ب. منزل

متزوج

مسلمة

الجنس

محمد هادي كمال يوسف فخافي

الطاقة سارية حتى ٢٠٢٩/٠٣/١٢

١١



MDT FORM

Patient Name	ريهام سمير صالح حسن	Age	27
Sex	F	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01120871695 01102187070
Menopausal Status	pre	Marital Status	married
ID	29705281200961	Medical ID	2654/2023
Date	30/9/2025	Reason of MDT	

Medical Report

Pathology: IDC Grade 2

ER: 8/8

PR: 8/8

HER2: +3

T N M

S.M:

Metsworkup:

Doctors' notes: Patient underwent Bx outside → revision → ER 0/8, PR 3/8, HER2 +ve
Underwent Rt completion mastectomy 10/2023 → 2 TCHP, continue 4 TC + Herceptin + PORT
Kept on tamoxifen + herceptin maintenance

Recommendations : For shifting to AI , Zoladex, Zometa every 6 months

MDT DECISION

Due to high risk status shift to AI (Femara) + Zoladex + Zometa/ 6 month for 5 years

Name	د/ احمد مرسى	د/ هاشم عبدالرحمن	د/ ابو بكر حجازي
Signature	د/ خالد عبد العزيز	Dr Haseel	
	30/9/2025	30/9/2025	



Patient Name	: الأستاذة / ريهام سمير صالح حسن محمد	Request No.	: 13009232
Patient No.	: 29705281200961	Request Date	: 30/10/2023
Path No.	: 3761/23	Verification Date	: 03/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 26 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer, (Up-front)
Nature of Specimen: Right completion mastectomy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 50% nuclear reaction of tumor cells.

Conclusion:

Right breast completion mastectomy:
Invasive duct carcinoma, Grade II with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Strong (score 3+).
- Ki67: Positive in 50% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

Dr.Marwa mahmoud

Printing Date : 03/01/2024 14:32:39

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mitr El Qadeima - Cairo, Egypt



Patient Name	الأستاذة / ريهام سمير صالح حسن محمد	Request No.	13009232
Patient No.	29705281200961	Request Date	30/10/2023
Path No.	3761/23	Verification Date	03/01/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	26 Y	Department	Out Patient
Referring Doc.			

Registered by Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy

Dr. Heba Hasan

Verified by :

Dr. Marwa mahmoud

Printing Date : 03/01/2024 14:32:39

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Total DLP (mGy.cm):

Patient Name: Riham Samir Saleh	Date : 29/05/2024
Exam : Item 1	Age : 27 Year(s)
Medical ID: MOSTAFA^BRAIN	Pat. ID: 41604

MRI EXAMINATION OF THE BRAIN

MRI TECHNIQUE:

- Sagittal T2WI
- Axial T1, T2 and FLAIR WIs
- Coronal T1WI
- Axial, sagittal and coronal T1 post contrast.

MRI FINDINGS:

- Normal size and configuration of the ventricular system.
- Normal size and parenchymal signal intensity pattern of the cerebral hemispheres.
- No shift of the midline structures.
- No intra or extra axial S.O.Ls.
- No extra-axial collections.
- Normal appearance of the cerebellum, brain stem and cranio-cervical junction.
- Bilateral maxillary and ethmoidal sinusitis.

OPINION:

- Normal contrast enhanced MRI of the brain.

Dr, Hala Sayed, MSc

Dr. Amr Farouk, M.D.



Patient Name: Riham Samir Saleh Hassan Mohamed	Date : 10/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 27 Year(s)
Medical ID: 2654/2023	Pat. ID: 41604

LEFT BREAST MAMMOGRAPHY WITH BILATERAL US

Technique

- LEFT breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right axillary, supra and infra regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Asmaa Zenhom

10/10/2024 2:16 PM

PAID 10128875

Pat. Age 26 Year(s)

Pat. Age 26 Year(s)

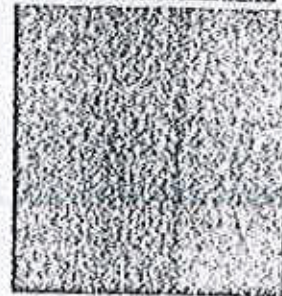
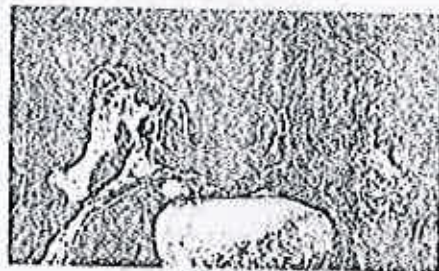
High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

- Axial T2
- Axial STIR
- Sagittal postcontrast 3D TFE of the right breast .
- Sagittal postcontrast 3D TFE of the left breast .
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction.

History of lumpectomy of pathologically proven breast carcinoma.

Right breast:

- Reduced right breast size ,postoperative change.
- Extreme fibroglandular tissue
- Moderate background parenchymal enhancement.
- Right UIQ postoperative changes are noted at 2-3 O'clock with fibrous scarring of high T2WIs and STIR showing homogeneous enhancement in the post contrast series. The kinetic data obtained from it indicates a benign pattern, continuous steady rise not followed by wash out (type 1 curves).
- Operative bed fat necrosis is also noted with low signal in STIR and high signal in T1WIs.
- Few right breast cysts of bright signal in T2WIs and STIR showing no enhancement in the post contrast series and measuring 5-13 mm in diameter.
- No residual operative bed pathologically enhancing lesions.
- No residual architecture distortion.
- Diffuse thickening and enhancement of the skin of the right breast ,most evident at the operative bed at the UIQ.



- Pathologically enlarged right internal mammary LN is seen at the level of 5 O'clock ,about 8.4 cm from the nipple ,measuring 19x10 mm in diameter showing restricted diffusion within and pathological enhancement in the post contrast series.
- Pathologically enlarged lymph node is seen at level III axillary LN groups ,measuring 25x9.7 mm in diameter with thickened cortex and obliterated fatty hilum.

Left breast:

- Extreme fibroglandular tissue
- Mild background parenchymal enhancement.
- Left breast few punctuate foci of parenchymal enhancement, the kinetic data obtained from them indicates a benign pattern, continuous steady rise not followed by wash out (type I curves).
- No architecture distortion.
- No breast solid or cystic lesions.
- Normal skin thickness and contour of the left breast.
- There is multiple likely non-specific axillary lymphadenopathy.

OPINION :

Right internal mammary and right axillary pathological lymphadenopathies, categorized as BIRADS 6, based on the kinetic and morphological data.

Right UIQ postoperative changes with no residual operative bed pathologically enhancing lesions.

Few right breast cysts.

Mild left mammary fibroadenosis ,categorized as BIRADS 2 ,based on the kinetic and morphological data.

Dr.S.Ali.MD

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category

Suspected Cancer Risk

BIRADS 0 : Non-informative study.

BIRADS 1 : Normal study.

BIRADS 2 : Benign findings.

BIRADS 3 : Probably benign.

0 %

0 %

2 %



Exam Information:

<u>Patient Name:</u> Reham Samer	<u>Date of birth:</u> 28/05/1997
<u>Arabic Name:</u> ريهام سمير	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 341972 <u>Patient HIS ID:</u> 123024054	<u>Exam Date:</u> 09/11/2023
<u>Written by:</u> Dr. Hoda Fathy	<u>Approved by:</u> Dr. Hoda Fathy

Nuclear Medicine Unit

وحدة الطب النووي

^{99m}Tc -MDP BONE SCAN

Brief history: 26 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP in the following manner:

Report:

- No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Homogeneous tracer distribution noted over axial and appendicular skeleton.
- Both kidneys are normally visualized.

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Thanks for reference